

may have a heart arrhythmia, so if you have antibodies, your baby will be monitored in pregnancy. Babies may show signs of some liver or kidney disfunction at birth. In most cases this will right itself within six months and babies generally show no long-lasting effects.

» Medicines and tests

Throughout your pregnancy, your obstetrician, as well as your SLE doctor, will arrange additional visits to assess you and your baby. You'll need to see your rheumatologist at least once every three months throughout. Take your medication, unless your doctors advise you otherwise. There is a much better outcome for your pregnancy and baby if your lupus is under control.

» Effects of pregnancy on your condition

Pregnancy shouldn't cause any extra complications for sufferers of SLE as long as it's under control at conception.

Multiple sclerosis

» What is it?

Multiple sclerosis (MS) damages nerve cells in the brain and spinal cord, affecting muscle movement, balance, and vision.

» Effects on your pregnancy, labor, and baby

MS seems to have no secondary effects for your pregnancy or baby. If MS has affected your ability to feel contractions in the muscles of your pelvis, you may need to have an assisted birth or a C-section. There is only a very small increased risk that your baby will inherit MS. You do have an increased risk of a relapse in pregnancy and after the birth.

» Medicines and tests

Consult your doctor about your medication. Many women with MS can continue to take steroids, particularly after the first trimester. You may have additional prenatal visits.

» Effects of pregnancy on your condition

Hormone levels may help reduce the rate of MS relapse during pregnancy, particularly during the third trimester. You may experience exacerbated symptoms of fatigue, back pain, bowel dysfunction, and bladder weakness, as well as an impaired sense of balance as your baby grows and your center of gravity shifts forward. Your increased risk of falls may mean that you need increased support when moving.

Sickle-cell disease

» What is it?

A group of genetic blood disorders, such as sickle-cell anemia and thalassemia. Affected hemoglobin in the red blood cells means they are unable to carry oxygen efficiently around the body, causing lack of energy, sudden attacks of pain, or ongoing pain when the tissues are starved of oxygen, and damage to internal organs over time.

» Effects on your pregnancy, labor, and baby

You have an increased risk of premature labor and a low birth weight baby. Your baby will carry the sickle-cell gene

(even if the baby's father has no sickle-cell gene); if the father is a carrier (without symptoms), your baby has a 50 percent chance of being born with sickle-cell disease (and a 50 percent chance of being a carrier). Your partner can be screened to figure out the risk of your baby inheriting the disease. You're at increased risk of preeclampsia, high blood pressure, stroke, breathing difficulties, and anemia. Your doctor may advise you to give birth in the hospital, where your baby will be attached to a fetal heart monitor during labor. During labor and delivery, you won't be offered every pain-relief medicine that other women are offered, but some analgesics should be fine.

» Medicines and tests

You will have lots of prenatal appointments, as well as regular visits to your hematologist. You may be advised to take extra folic acid and have a course of antibiotics to help protect you against infection. You may be advised to take a small daily dosage of aspirin after your first trimester to reduce the risk of high blood pressure. You will be offered an early ultrasound at between 7 and 9 weeks to assess the viability of your pregnancy, and then growth ultrasounds for your baby once a month in your last trimester.

» Effects of pregnancy on your condition

SCD symptoms may worsen during pregnancy. Sickness can exacerbate them—consult your doctor about the best way to minimize the effects.

Cystic fibrosis

» What is it?

Known as CF, this genetic condition causes thick mucus to build up in the lungs and digestive system, making it difficult to breathe and get nutrients from food.

» Effects on your pregnancy, labor, and baby

You will probably be advised that a lung function (FEV1) of 60 percent is necessary before you try to become pregnant. Pregnancy is best planned. The likelihood of having a healthy pregnancy and baby when you have CF is improving all the time. Your doctors can help you get a good outcome. You will pass the CF gene to your baby, who will become a carrier. He has a 50 percent chance of developing the disease only if his father is a carrier too. You have a higher risk of gestational diabetes, your baby may be small, and there is an increased risk of premature labor, miscarriage, and stillbirth.

» Medicines and tests

Most medicines used in CF, including antibiotics, are safe to use in pregnancy. You can continue any medication unless your CF specialist advises otherwise. You will see your doctor with your CF specialist. Both will monitor your nutrition and weight gain. In some cases you may need additional ultrasounds to

Phenylketonuria

» What is it?

In this genetic condition, the body cannot break down phenylalanine, which can cause brain and nerve damage.

» Risks to your pregnancy, labor, and baby

High levels of phenylalanine can cross the placenta and cause serious defects and even fatality in your unborn baby. You will need to adhere strictly to a diet regimen in order to keep levels of blood phenylalanine within the range of 100–250 umol/L throughout the course of your pregnancy. Your baby could have low birth weight. Your baby will only inherit PKU if her father is a carrier, which in the US gives him about a 1 in 200 chance.

» Medicines and tests

You will need to have blood tests up to three times a week and you will see your PKU specialist up to once a month. You may be asked to go for additional growth ultrasounds.

» Effects of pregnancy on your condition

Your protein-controlled diet will be strict. As your baby grows he will use up protein, and later his liver may produce the enzymes you cannot. As a result, the amount of protein you eat to keep your blood phenylalanine at the correct levels will increase over the course of the pregnancy—all within the strict diet prescribed by your dietician in response to your blood tests.

check the development and growth of your baby. Check with your doctor if you can continue to take pain-relief medication. Consult your CF specialist before trying to conceive.

» Effects of pregnancy on your condition

The symptoms of CF might worsen. You might find that you have trouble with breathing as your uterus expands during your pregnancy. You may also experience malnutrition as your body struggles to absorb enough nutrients. Nasal tube feeding may be advised.



Good lung function This is a key factor in having a safe pregnancy and healthy baby. Your doctors will monitor your lung function throughout your pregnancy.